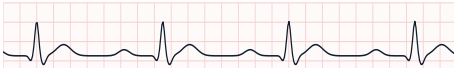
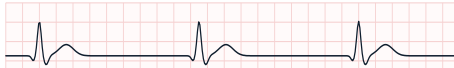
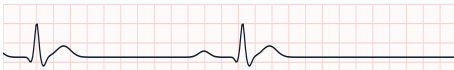
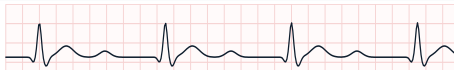
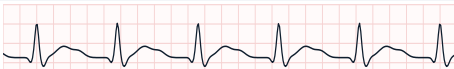
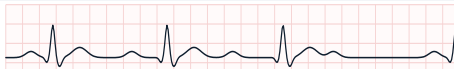
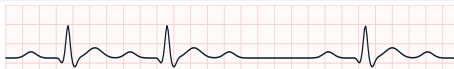
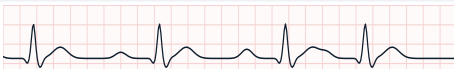
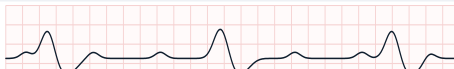
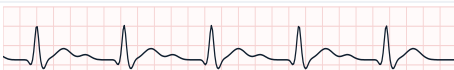
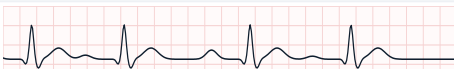
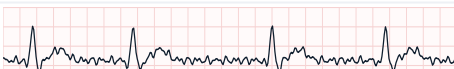
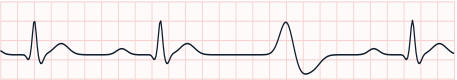
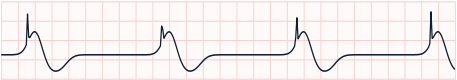
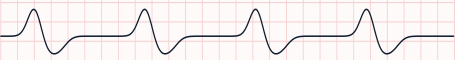
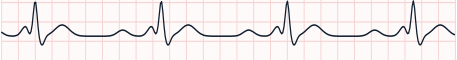
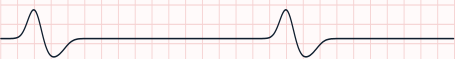
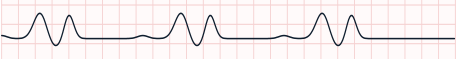
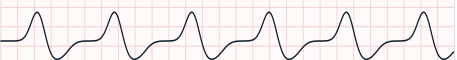
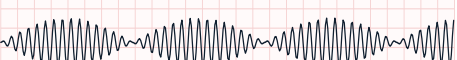


ACLS Cardiac Rhythm Recognition — Supraventricular

Sinus & atrial rhythms · junctional & AV blocks · recognize the pattern, then treat per algorithm

Renal Care Unit · clinician reference

SINUS & ATRIAL		JUNCTIONAL & AV BLOCKS		
Normal Sinus Rhythm P before every QRS; regular; 60–100		Junctional Escape No/inverted P; 40–60		
Sinus Bradycardia Regular, rate < 60		1st-degree AV Block Constant long PR (>0.20 s)		
Sinus Tachycardia Regular, rate > 100		2nd-deg Mobitz I (Wenckebach) PR lengthens, then dropped beat		
Sinus Arrhythmia Rate varies with respiration		2nd-deg Mobitz II Constant PR; sudden dropped QRS		PERI-ARREST
Premature Atrial Complex Early P, abnormal morphology		3rd-degree (Complete) P and QRS independent		PERI-ARREST
Atrial Tachycardia Fast, abnormal P-wave axis		SHOCKABLE VF · pulseless VTNON-SHOCK asystole · PEA PERI-ARREST unstable — treat per algorithm. Strips are schematic single-lead illustrations for pattern recognition, not measured tracings.		
Multifocal Atrial Tachy ≥3 P morphologies; irregular				
SVT (AVNRT) Narrow, regular, ~150–250; no P			PERI-ARREST	
Atrial Flutter Saw-tooth flutter waves				
Atrial Fibrillation Irregularly irregular; no P				

VENTRICULAR & ARREST			PACED & SPECIAL		
Premature Ventricular Complex Early, wide, bizarre QRS			Paced Rhythm Pacer spike before each complex		
Accel. Idioventricular (AIVR) Wide, regular, 40–120; no P			WPW / Pre-excitation Short PR; delta wave		
Idioventricular / Agonal Slow, wide, dying rhythm			Hyperkalemia (dialysis) Peaked T → wide QRS → sine wave		PERI-ARREST
Monomorphic VT Wide, regular, fast		SHOCKABLE	SHOCKABLE VF · pulseless VT NON-SHOCK asystole · PEA PERI-ARREST unstable — treat per algorithm. Strips are schematic single-lead illustrations for pattern recognition, not measured tracings.		
Torsades de Pointes Polymorphic; twists around baseline		SHOCKABLE			
Ventricular Fibrillation Chaotic; no organized complexes		SHOCKABLE			
Pulseless VT Wide regular VT, NO pulse		SHOCKABLE			
Asystole Flat line; confirm in 2 leads		NON-SHOCK			
PEA Organized rhythm, NO pulse		NON-SHOCK			

Schematic ECG reference for trained personnel — for pattern recognition, not diagnosis. Always correlate with the patient, a 12-lead ECG, and the responding physician's judgment. Verify against your local protocols.